

1. Administrative & Study Identification

Full Study Title: A phase III, randomized, double-blind, placebo-controlled, multi-center, global study to explore the efficacy and safety of volrustomig in women with high-risk LACC (FIGO 2018 stage IIIA to IVA cervical cancer) who have not progressed following platinum-based CCRT **Short Title/Acronym:** eVOLVE-Cervical (Study of Volrustomig in Women With High Risk Locally Advanced Cervical Cancer) **Protocol Number:** D7984C00002 **NCT Number:** NCT06079671 **Sponsor Name:** AstraZeneca **Investigational Product:** Volrustomig (MEDI5752, PD-1/CTLA-4 bispecific immunotherapy) vs. Placebo **Phase of Development:** Phase III **Medical Monitor:** AstraZeneca Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the efficacy of volrustomig compared to placebo by assessing Progression-Free Survival (PFS) in women with high-risk locally advanced cervical cancer (LACC). **Secondary Objectives:** To evaluate Overall Survival (OS), Objective Response Rate (ORR), and Duration of Response (DoR).

2.2 Background & Rationale To address the clinical need for improved maintenance therapies in women with high-risk LACC (FIGO 2018 stage IIIA to IVA) who have not progressed following platinum-based concurrent chemoradiotherapy (CCRT). Volrustomig, a bispecific antibody targeting both PD-1 and CTLA-4, aims to enhance the anti-tumor immune response and extend progression-free survival compared to placebo in this high-risk population.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Placebo-controlled **Blinding:** Double-blind **Randomization:** 1:1 ratio (Volrustomig : Placebo)

3.2 Planned Enrollment & Duration Target \$N\$: 800 participants **Number of Sites:** Multi-center, global **Estimated Study Start:** 22-Sep-2023 **Estimated Primary Completion:** 26-Nov-2027

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Female, aged ≥ 15 years at the time of screening. Body weight ≥ 35 kg. 2. Histologically documented

FIGO 2018 Stage IIIA to IVA cervical adenocarcinoma, cervical squamous carcinoma, or cervical adenosquamous carcinoma, with no evidence of metastatic disease. 3. Initial staging procedures performed no more than 56 days prior to the first dose of CCRT, and provision of FFPE tumor sample to assess PD-L1 expression.

4.2 Exclusion Criteria 1. Diagnosis of small cell (neuroendocrine) or mucinous adenocarcinoma of cervical cancer, or evidence of metastatic disease. 2. Intent to administer a fertility-sparing treatment regimen, history of organ transplant, or allogeneic stem cell transplant. 3. History of active primary immunodeficiency or active/prior documented autoimmune or inflammatory disorders.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Volrustomig or matching placebo administered in dedicated treatment cycles. **Route of Administration:** Intravenous infusion. **Duration of Treatment:** Continued until disease progression, unacceptable toxicity, or other protocol-defined discontinuation criteria are met.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care medications. **Prohibited:** Any prior (besides prior CCRT) or concurrent treatment for cervical cancer; exposure to immune-mediated therapy prior to the study; live attenuated vaccine within 30 days prior to the first dose of the study intervention.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Tumor Sample Collection, Imaging
Baseline	Day 0	Randomization (1:1), First Dose of Volrustomig/Placebo
Interim	Per Protocol	Safety Labs, Efficacy Assessment (Tumor Imaging), AE Monitoring
End of Study	Until Progression	Final Vital Signs, Exit Interview, Follow-up for Survival

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Progression-Free Survival (PFS) (Investigator-assessed, ITT). **Measurement Method:** Radiological assessment according to RECIST v1.1.

7.2 Secondary & Exploratory Endpoints 1. Overall Survival (OS) 2. Objective Response Rate (ORR) 3. Duration of Response (DoR)

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring of adverse events (e.g., hepatotoxicity, immune-related AEs) via regular patient assessments and lab tests. **Serious Adverse Events (SAEs):** Standard oncology trial 24-hour reporting timeline for SAEs. **Data Monitoring Committee (DMC):** Independent external DMC established to monitor safety and efficacy signals globally.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for primary efficacy (PFS) and secondary endpoints (OS, ORR, DoR). Safety Analysis Set for all patients receiving at least one dose of study drug. **Sample Size Justification:** $N=800$ statistically powered to detect a significant hazard ratio improvement in PFS and OS compared to placebo. **Handling of Missing Data:** Censoring rules defined according to FDA/EMA oncology guidelines for time-to-event endpoints.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Regular onsite and remote monitoring visits for Source Data Verification (SDV). **Audit Trail:** Robust eCRF systems with comprehensive audit trails and data clarification form mechanisms.

11. Study Identifiers & Governance

Primary ID: D7984C00002 **Registry Number:** NCT06079671 **Sponsor/Lead Organization:** AstraZeneca **Study Title:** eVOLVE-Cervical **Official Regulatory Title:** A phase III, randomized, double-blind, placebo-controlled, multi-center, global study to explore the efficacy and safety of volrustomig in women with high-risk LACC (FIGO 2018 stage IIIA to IVA cervical cancer) who have not progressed following platinum-based CCRT

12. Clinical Framework (Cervical Cancer Specific)

Primary Condition: High-Risk Locally Advanced Cervical Cancer (LACC) **Diagnosis Threshold:** FIGO 2018 Stage IIIA to IVA **Medical Methodology:** Definitive platinum-based concurrent chemoradiotherapy (CCRT) followed by bispecific immunotherapy maintenance **Optimization Target:** Sustained progression-free survival (PFS) via PD-1/CTLA-4 dual immune-checkpoint blockade

13. Study Procedures & Methodology

Management Pathway: Standard oncology clinical pathway for LACC evaluation and follow-up **Education Protocol (HCP):** Site initiation visits and continuous Good Clinical Practice (GCP)/ protocol training **Education Protocol (Patient):** Informed consent, adverse event awareness, and reporting procedures **Quality Audit Mechanism:** Routine central monitoring and independent auditing of investigator radiological assessments

14. Observation & Follow-Up Schedule

Total Duration: Follow-up until progression, death, or trial completion (Est. 2030) **Onsite Visit Cadence:** Standard oncology infusion and imaging intervals (e.g., every 3-4 weeks for dosing, imaging Q12W) **Remote Monitoring Frequency:** Intermittent monitoring throughout the maintenance phase **Checklist Review Interval:** Regular tumor assessments per RECIST criteria

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				